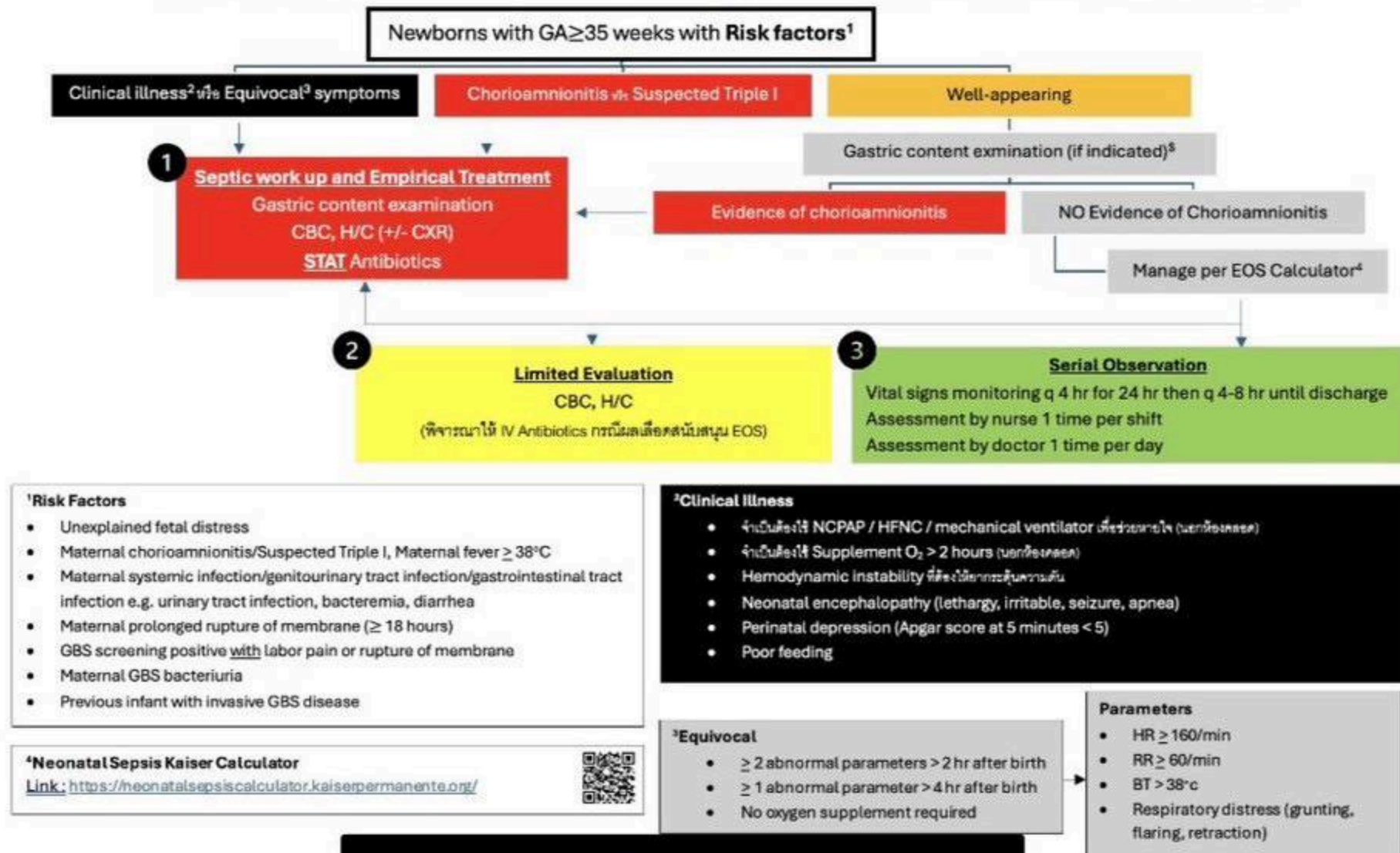


Decision Guideline to Evaluate Early Onset Neonatal Sepsis in Newborn with GA $\geq$ 35 weeks with Risk Factors

แนวทาง EOS KCMH

Decision Guideline to Evaluate Early Onset Neonatal Sepsis in Newborn with GA < 35 weeks

Newborn < 35 wk	Lower risk of EOS Infants met ALL criteria of the following	- คลอดด้วยข้อบ่งชี้ทางสูติศาสตร์ เช่น severe pre-eclampsia, severe fetal growth restriction - คลอดด้วยวิธีการ Cesarean section - มารดาไม่มี Labor pain และไม่มี rupture of membrane	Limited evaluation พิจารณาส่งตรวจ CBC หากทารกจำเป็นต้อง IV access
	Higher Risk of EOS ANY of the following	- Cervical incompetence - Premature rupture of membrane - Preterm labor pain - Maternal fever, suspected Triple I, chorioamnionitis - Unexplained non-reassuring fetal status/distress - ทารกป่วย	Septic work up and Empirical Treatment Gastric content examination CBC, H/C (+/- CXR) STAT Antibiotics

⁵Gastric Content Examination

POSITIVE gastric content examination

- Wet smear revealed ≥ 5 WBC/hpf
- Evidence of positive gram stain OR culture of pathogen

Laboratory Evidence of Chorioamnionitis

Managing infants as Chorioamnionitis is recommended

Who SHOULD be screened with gastric content examination? → Born from pregnant women who are diagnosed with OR high-risk for chorioamnionitis

- Sick infants that infection cannot be ruled out
- Maternal chorioamnionitis/intra-uterine infection, inflammation or both (Triple I), maternal fever $\geq 38^\circ\text{C}$
- Maternal systemic infection/genitourinary tract infection/gastrointestinal tract infection e.g. urinary tract infection, bacteremia, diarrhea
- Maternal prolonged rupture of membrane (≥ 18 hours)
- Unexplained fetal distress
- Preterm < 35 weeks with preterm labor or rupture of membrane